

TOBACCO SMOKING

Assess the patient who smokes tobacco currently or recently stopped

Assess	When to assess	Note
Symptoms	Every visit	- Ask about symptoms that might suggest cancer: cough/difficulty breathing ↗ 38, urinary symptoms ↗ 59 or weight loss ↗ 23. - Ask about symptoms of CVD¹: chest pain ↗ 37, leg pain ↗ 65, new sudden onset of any of: asymmetric weakness of face, arm or leg; numbness, difficulty speaking or visual disturbance ↗ 136. - Manage other symptoms as on symptom pages.
Tobacco use	Every visit	- Ask about number of cigarettes per day and what activities patient does while smoking. - If recently stopped, praise patient and encourage to avoid re-starting: reinforce advice about risks, benefits, distraction techniques and support helpline/groups available ↗ 178. - Ask about previous attempt at stopping: review what helped and why attempt failed, address reason for relapse before another quit attempt.
Stressors	Every visit	Help identify the domestic, social and work factors contributing to smoking tobacco. If low mood, stress or anxiety ↗ 86.
COPD	At diagnosis	If difficulty breathing when walking fast/up a hill, consider COPD ↗ 123. If known COPD ↗ 126.
CVD risk	At diagnosis	Assess CVD risk ↗ 127.

Advise the patient who smokes tobacco

- Ask if patient is willing to discuss tobacco smoking. For tips on how to communicate effectively ↗ 176. Support the patient to make a change ↗ 177.
- Advise patient that stopping tobacco smoking is the most important action s/he can take to improve health, quality of life and increase life expectancy.
- Explain that nicotine is very addictive and stopping can cause withdrawal symptoms: increased appetite, mood changes, difficulty sleeping/concentrating, irritability, anxiety, restlessness. These should improve after 2-4 weeks.
- Advise that most smokers make several attempts to stop before they are successful.
- If patient is pregnant or breastfeeding, stress the importance of stopping for baby's health.
- Ask if patient is willing/ready to stop smoking tobacco and give the advice below:

If patient is not ready to stop in the next month

- Discuss risks to patient (worsening asthma, infertility, heart attack, stroke, COPD, cancer) to spouse (lung cancer, heart disease) and to children (low birth weight, asthma, respiratory infections).
- Help identify benefits of stopping tobacco smoking like saving money, improved health, taste, sense of smell and appearance and being a positive role model for children.
- Help identify barriers to stopping tobacco smoking and possible solutions.
- Ask if patient is ready to stop smoking tobacco in the next month. If not ready to stop, encourage patient to return, use helpline ↗ 178 or support group when ready to stop.

If patient is ready to stop in the next month

- Help patient plan: set date to stop within 2 weeks, seek support from family and friends, support group or helpline ↗ 178, avoid/manage situations associated with smoking and remove cigarettes, matches, and ashtrays. Help manage cravings using a stepwise approach, starting with step 1. If urge does not subside, move on to next step.
 - Step 1: delay as long as you can.
 - Step 2: take a deep breath and blow out slowly (repeat 10 times).
 - Step 3: drink water as an alternative to tobacco smoking.
 - Step 4: distract yourself with reading a book, going for a walk, listening to music, watching TV or other hobby.
- Offer referral for counselling especially if failed previous attempt at stopping, previous depression or alcohol misuse.

Review patient within the first week of stopping tobacco smoking and then as needed.

¹Cardiovascular disease (CVD) includes ischaemic heart disease, peripheral vascular disease and stroke/TIA.